

Hospital letter

NHS FOUNDATION TRUST
CARDIOLOGY DEPARTMENT
St George's University Hospital
123 Hospital Road
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Telephone: 01234 555555

Date: 28 July 2025

Private and Confidential

Dr Sarah Williams
Riverside Medical Practice
123 High Street
Anytown
AB1 2CD

Dear Dr Williams,

Re: Michael Thompson

DOB: 14 June 1965

NHS No: 987 654 3210 (Fictional)

Diagnosis:

- Ischaemic Heart Disease
- Previous Myocardial Infarction
- Stable Angina
- Osteoarthritis
- Chronic Back Pain
- Depression and Anxiety

Thank you for referring Mr Thompson for cardiology follow-up following his recent admission with chest pain.

Mr Thompson attended clinic today accompanied by his partner. He reports experiencing intermittent episodes of exertional chest discomfort over the preceding twelve months. These symptoms are typically brought on by walking moderate distances, climbing inclines or undertaking prolonged physical activity, and generally resolve with rest.

As you are aware, Mr Thompson was admitted to hospital on 18 June 2025 following an episode of prolonged central chest pain associated with shortness of breath. Serial cardiac enzymes and ECG monitoring showed no evidence of an acute myocardial infarction. He was discharged following a period of observation and outpatient follow-up was arranged.

At today's consultation he describes ongoing limitations due to a combination of cardiovascular disease, chronic back pain and osteoarthritis affecting multiple joints. He reports that these conditions significantly restrict his mobility. He now regularly uses a walking stick when outdoors and states that he needs to stop and rest after walking short distances due to a combination of chest discomfort, breathlessness and joint pain.

Mr Thompson also discussed the impact that his physical health has had on his mental wellbeing. He reports ongoing symptoms of depression and increasing anxiety regarding his health. He described concerns about becoming unwell whilst away from home and reports

that he increasingly relies upon support from his partner when attending appointments or travelling to unfamiliar locations.

Examination today revealed a patient who was comfortable at rest with no evidence of acute cardiac symptoms.

Blood pressure: 134/78 mmHg

Pulse: 62 beats per minute, regular

Cardiovascular examination was unremarkable.

Recent investigations were reviewed:

ECG:

Sinus rhythm with no acute ischaemic changes.

Echocardiogram:

Mild left ventricular impairment consistent with previous myocardial infarction. No significant valvular abnormalities identified.

Blood Tests:

Stable renal function and cholesterol profile.

Assessment

Mr Thompson's symptoms are most consistent with stable angina in the context of known ischaemic heart disease. There is currently no evidence of unstable cardiac disease. However, he continues to experience significant functional limitation due to a combination of his cardiac condition and chronic musculoskeletal problems.

Plan

- Continue current medication regimen including GTN spray, Aspirin, Atenolol, Isosorbide Mononitrate and Atorvastatin.
- Continue management of chronic pain via primary care.
- Encourage pacing of activities and avoidance of overexertion.
- Continue monitoring of mental health symptoms with GP.
- Review in Cardiology Clinic in 12 months unless symptoms deteriorate earlier.

It is my opinion that Mr Thompson's chronic health conditions continue to have a substantial impact on his functional ability and mobility. Given the chronic nature of his cardiac and musculoskeletal conditions, significant improvement is unlikely in the short term.

Thank you for allowing me to participate in Mr Thompson's care. Please do not hesitate to contact me should you require any further information.

Yours sincerely,

Dr James Patel

Consultant Cardiologist

GMC No: 6054321 (Fictional)

Cardiology Department

St George's University Hospital

Cc: Michael Thompson